

CLUSTERING & CLASSIFYING CREDIT CARD CUSTOMERS

STAT 101 Final Project
Part 1

PREPARED BY:

Kyle Maher

Executive Summary

The number of clusters was determined based on whether an additional cluster identified a group with characteristics significantly different from another cluster. Five clusters were identified. Cluster 1 is the largest. Clusters 2-5 share all but one characteristic with Cluster 1. Cluster 2 has a higher number of credit cards. Cluster 3 has a lower average balance. Cluster 4 has a higher number of open bank accounts. Cluster 5 owns more houses.

A random bootstrap forest model was created to predict whether an individual would accept the credit card offer. Due to the unbalanced nature of the data set a model created without any changes to the data completely missed the small subset of individuals who accepted the credit card offer. As a result, synthetic data mimicking the minority group was inserted into the data to balance the groups. The resulting model correctly predicted whether an individual would accept the offer 94.3% of the time when tested against new data.

Details of Analysis and Recommendations

Data Preparation:

ADDRESSING MISSING VALUES:

24 rows contained no data for "Q1 Balance," "Q2 Balance," "Q3 Balance," "Q4 Balance," and "Average Balance." All 24 rows were excluded. The excluded data included 2 individuals who accepted the offer and 22 individuals who rejected the offer.

CHECKING OUTLIERS:

Significant yet reasonable outliers identified in "# Bank Accounts Open," "# Credit Cards Held," "Household Size," "Q1 Balance," "Q2 Balance," "Q3 Balance," "Q4 Balance," and "Average Balance."

UNCORRELATED DATA REMOVAL:

"Customer Number" was excluded from all analysis processes.

CORRELATED DATA REMOVAL:

Since the "Average Balance" and Q1, Q2, Q3, and Q4 balances are correlated, and use of correlated parameters can introduce excess noise into the data set, it makes sense in some cases to only use the "Average Balance" column.

For Clustering Analysis: Only "Average Balance" used.

For Prediction Model: All balance related columns used.

Exploratory Data Analysis:

STRONGEST CORRELATIONS:

"Average Balance" & "Q2 Balance" (.8003)

"Average Balance" & "Q3 Balance" (.7527)

	Average Balance	Q1 Balance	Q2 Balance	Q3 Balance	Q4 Balance
Average Balance	1.0000	0.4815	0.8003	0.7527	0.5724
Q1 Balance	0.4815	1.0000	0.4831	-0.0610	-0.2373
Q2 Balance	0.8003	0.4831	1.0000	0.4877	0.1690
Q3 Balance	0.7527	-0.0610	0.4877	1.0000	0.5653
Q4 Balance	0.5724	-0.2373	0.1690	0.5653	1.0000

Results of Customer Segmentation Analysis:

CONSIDERING DIFFERENT NUMBERS OF CLUSTERS:

The addition of a fifth cluster adds a unique group who hold a higher number of credit cards. Additionally, the fifth cluster is similar in size to the other clusters. The addition of a sixth cluster adds a smaller group with no significant differences from Cluster 4. Since the addition of a fifth cluster added a group with at least one unique characteristic, and the addition of a sixth cluster did not, five clusters were used.

CLUSTER BREAKDOWN:

Cluster 1 is the largest cluster and shares many characteristics with the other clusters. Clusters 2-5 share all but one characteristic with Cluster 1.

Cluster 2 includes individuals who hold a higher number of credit cards (with a mean of 2.98).

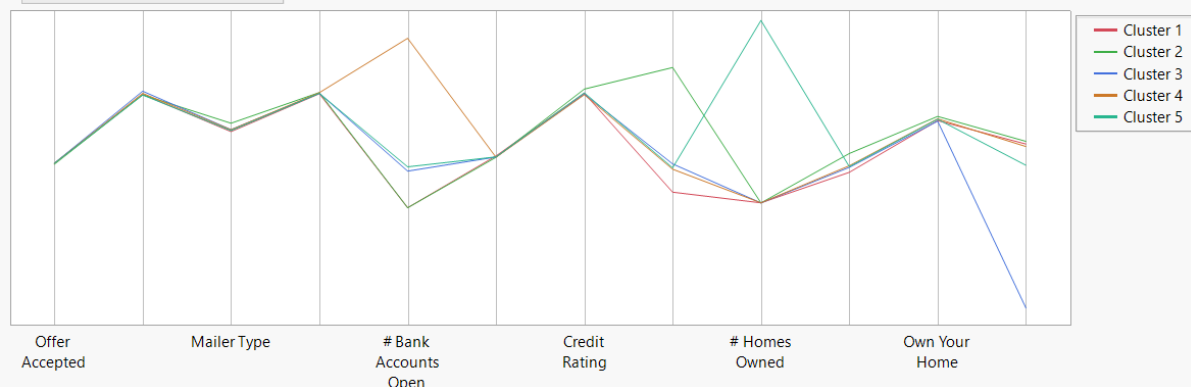
Cluster 3 includes individuals who have a lower average balance (with a mean of 276.37).

Cluster 4 includes individuals who have a higher number of open bank accounts (with a mean of 2.08).

Cluster 5 includes individuals who own a greater number of houses (with a mean of 2.05).

Cluster	Count	Offer Accepted	Reward	Mailer Type	Income Level	# Bank Accounts Open	Overdraft Protection	Credit Rating	# Credit Cards Held	# Homes Owned	Household Size	Own Your Home	Average Balance
1	7590	0.07016	1.80075	0.75144	1.70864	1.00000	0.20510	1.82735	1.64	1.00	3.43	0.95	1052.08
2	1836	0.06726	1.79588	0.80793	1.71975	1.00000	0.19830	1.87332	2.98	1.00	3.71	0.98	1064.64
3	2121	0.07088	1.83628	0.76458	1.71313	1.23338	0.20005	1.81773	1.94	1.00	3.50	0.95	276.37
4	2958	0.07034	1.81231	0.76401	1.71896	2.08080	0.19857	1.81119	1.89	1.00	3.53	0.96	1040.35
5	3471	0.07000	1.79364	0.75831	1.71196	1.26073	0.20055	1.83214	1.90	2.05	3.51	0.96	951.66

Cluster Standard Deviations



RECOMMENDATIONS:

As the largest Cluster, general credit card offers should be designed to fit the needs of individuals within Cluster 1. Alternative offers could be targeted towards individuals with more credit cards, individuals who have a smaller balance, individuals who have more bank accounts, or individuals who own more houses. Offers that try to appeal to a specific audience by altering Reward, Mailer Type, Income Level, Overdraft Protection, Household Size, or Home Ownership may be less effective unless used in conjunction with offers to target the individuals within clusters 2-5.

Results of Predictive Model Analysis:

MODIFYING THE DATASET:

Creating a model to predict offer acceptance is complicated by the unbalanced nature of the data set. The offer accepted column contains a minority class with only 5.7% of individuals within the dataset accepting the offer. To balance the data set a Synthetic Minority Oversampling Technique (SMOTE) was conducted. The JMP Imbalanced Classification Add-In used can be found at <https://community.jmp.com/t5/JMP-Add-Ins/Imbalanced-Classification-Add-In/ta-p/304836>. Within the add-in, the "SOMTE Observations" technique was used to balance the sizes of the majority and minority groups. After adding synthetic data with SMOTE, 39.7% of individuals accepted the offer.

SPAM VS HAM

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Part 2

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Kyle Maher

ETHICS ANALYSIS

STAT 101 Final Project
Part 3

PREPARED BY:

Kyle Maher

Skin cancer data is difficult to find, likely due to privacy reasons, so I am working with the following assumption.

ASSUMPTION: Dermatologists diagnose about the same number of skin abnormalities as cancerous and non-cancerous (allows for balanced comparison of percentages).

CONSIDERATION 1:

Would it be ethical to entirely remove physicians from the diagnosis procedure and leave the diagnosis entirely to data science methods?

Confusion matrix for dermatologists diagnosing skin cancer:

Actual	Diagnosed as Cancerous	Diagnosed as Non-Cancerous
Cancerous	88.9%	11.1%
Non-Cancerous	24.3%	75.7%

Confusion matrix for data science methods:

Actual	Diagnosed as Cancerous	Diagnosed as Non-Cancerous
Cancerous	90.9%	9.1%
Non-Cancerous	44.3%	55.7%

It is ethical to switch to only Data Science methods if more lives are saved. When switching, an additional 2% of cases are correctly classified as cancerous. This increase in correct classification results in more individuals with cancer getting treated, saving lives. Although an additional 20% of cases are classified as cancerous when they are benign, the cancer treatment without cancer is likely less deadly than the cancer without any treatment. The extent to which that is true determines if the switch is ethical. If the mortality rate of the average treatment is less than 1/10th of the mortality rate of the average cancer, more lives would be saved by switching to Data Science methods. In other words, if someone with cancer and no treatment is over ten times more likely to die than someone with no cancer still getting cancer treatment, then it is ethical to switch to only Data Science methods.

CONSIDERATION 2:

Would it be ethical to entirely remove physicians from the diagnosis procedure and leave the diagnosis entirely to data science methods?

Confusion matrix for dermatologists diagnosing skin cancer:

Actual	Diagnosed as Cancerous	Diagnosed as Non-Cancerous
Cancerous	88.9%	11.1%
Non-Cancerous	24.3%	75.7%

Confusion matrix for data science methods:

Actual	Diagnosed as Cancerous	Diagnosed as Non-Cancerous
Cancerous	78.9%	21.1%
Non-Cancerous	4.3%	95.7%

It is ethical to switch to only Data Science methods if more lives are saved. When switching, an additional 10% of cases are incorrectly classified as non-cancerous. This incorrect classification results in more individuals with cancer not getting treated, costing lives. Although an additional 20% of cases are correctly classified as non-cancerous, cancer without any treatment is likely more deadly than the cancer treatment without cancer. If the mortality rate of the average treatment is less than half of the mortality rate of the average cancer, fewer lives would be saved by switching to Data Science methods. In other words, if someone with cancer and no treatment is over twice as likely to die than someone with no cancer still getting cancer treatment, then it is ethical to continue letting dermatologists diagnose skin abnormalities.

CONSIDERATION 3:

How does your opinion on this question compare to your opinion regarding letting automobiles be entirely self-driving?

If a self-driving car is less likely to get into a fatal accident than the average 18-year-old, living in the Midwest, with 2.5 years of experience, then I am perfectly comfortable letting the car drive itself. While everyone might not hold the same opinion, applied to a population, if the self-driving car is less likely to get into a fatal crash than the average individual in the population, it is ethical to switch to entirely self-driving cars. The main challenge is proving that that is or is not the case. The central theme between my opinion on the ethics of self-driving cars and using Data Science to diagnose skin cancer is the idea that whichever option will reduce fatalities is more ethical.